

Obsessive-Compulsive Disorder (OCD)

Obsessive-compulsive disorder (OCD) is a chronic mental disorder characterized by recurrent and distressing thoughts, urges, or mental images that the individual has difficulty controlling, together with repetitive behaviors or mental acts developed in response to them. OCD is not merely “liking to be organized” or placing excessive importance on something. The main problem is that unwanted thoughts repeatedly arise in the individual’s mind, and the individual feels compelled to perform certain behaviors in order to reduce the anxiety, fear, disgust, or doubt caused by these thoughts. Over time, this cycle can significantly affect the individual’s daily life, education, work, and relationships.

Obsessions in OCD are unwanted thoughts, urges, or images that enter the individual’s mind despite not being wanted and cause significant distress. The most common include fear of germs or contamination, fear of harming oneself or another person, concern about being responsible for an event, fear of making a mistake, a need for symmetry and order, unwanted sexual thoughts, and intense anxiety concerning religious or moral issues. In addition, obsessions related to relationships, identity, past events, or existential issues may also occur. These thoughts are generally inconsistent with the individual’s values and desires; that is, they are ego-dystonic. The individual does not want to have these thoughts and often recognizes that they are excessive or irrational.

Compulsions, on the other hand, are repetitive behaviors or mental acts performed with the aim of reducing the distress caused by obsessions or preventing a feared event from occurring. Examples include excessive handwashing and cleaning, repeatedly checking whether a door is locked or a stove is turned off, arranging objects in a specific order, counting, praying, mentally repeating words, seeking reassurance, and repeatedly reviewing events mentally. Avoidance behaviors may also be part of this cycle. A compulsion does not provide a permanent solution; it generally provides only temporary relief. Following this relief, the obsession reappears, and the individual turns to the compulsion again. Thus, a cycle develops in the form of obsession → anxiety/distress → compulsion → temporary relief → re-emergence of the obsession.

It is important to note that not every recurrent thought or behavior indicates OCD. People may occasionally worry about becoming ill, making a mistake, or something bad happening to their loved ones; they may also repeatedly check some of their behaviors. In OCD, however, these thoughts and behaviors are much more intense and persistent. They generally take more than one hour per day, cause significant distress, or interfere with important areas of daily life. The individual often feels compelled to perform the behavior even though they do not

want to do so. Therefore, compulsions in OCD differ from habits that the individual genuinely enjoys or prefers.

Symptoms of OCD may begin during childhood, adolescence, or young adulthood. Symptoms may change over time; they may decrease during certain periods and become significantly worse during stressful times. Particularly in children, the individual may not recognize that their behaviors are unusual and may fear that something bad will happen if they do not perform their rituals. Therefore, it may be important for parents or teachers to recognize these symptoms. The exact cause of OCD is not fully known. However, genetic predisposition, differences in certain brain regions and networks, temperament characteristics, and some childhood traumas are considered among the possible risk factors. Tic disorders, Tourette syndrome, depression, anxiety disorders, and certain other mental conditions may accompany OCD.

An important characteristic of OCD is that early recognition and treatment are important. Research indicates that a prolonged period of untreated illness may be associated with worsening symptom severity, the development of additional problems, and greater difficulty in responding to treatment. Despite this, accurate diagnosis may be delayed for years because of stigma, misunderstanding of symptoms, and the individual’s

reluctance to share their thoughts. Early assessment, particularly during childhood and adolescence, is important for the individual, their family, and teachers to recognize OCD and obtain appropriate support.

OCD is treatable, and psychotherapy and pharmacological treatment form the basis of treatment. In particular, Exposure and Response Prevention (ERP), a form of cognitive behavioral therapy, helps individuals learn to confront, in a controlled manner, situations that they fear or that trigger their obsessions and subsequently refrain from performing compulsive behaviors. In addition, medications from the selective serotonin reuptake inhibitor (SSRI) group, which target the serotonin system, may be used. In some individuals, psychotherapy and medication may be administered together. In severe cases that do not respond adequately to treatment, more advanced methods, such as brain stimulation, may be considered.

In conclusion, OCD is much more than being “obsessive” in the everyday sense and is a disorder that can significantly affect an individual’s life. At the core of the disorder are not only unwanted thoughts but also the individual’s feeling compelled to engage in repetitive behaviors or mental rituals in order to reduce the distress caused by these thoughts. With early recognition of this cycle, accurate assessment, and appropriate treatment, it may be possible for the individual to regain control over their life.

References:

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